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PATIENT DISCHARGE SUMMARY

Patient & Admission Details

Patient Name:	Margaret Eleanor Thornton	NHS Number:	485 777 3321
Date of Birth:	14 March 1952 (Age: 72)	Gender:	Female
Address:	42 Westbourne Terrace, London W2 6QA	GP:	Dr. Priya Mehta, Paddington Surgery
Admission Date:	09 November 2024 14:32	Discharge Date:	15 November 2024 10:15
Ward:	Hartley Ward 6B	Consultant:	Mr. James R. Hadfield, FRCS
Admission Type:	Emergency — via A&E;	Discharge Destination:	Home with community nursing

Diagnoses

#	Diagnosis	ICD-10	Status
1	Type 2 Diabetes Mellitus — uncontrolled	E11.65	Primary
2	Hypertensive Heart Disease with Heart Failure	I11.0	Secondary
3	Stage 3 Chronic Kidney Disease (CKD)	N18.3	Comorbidity
4	Community-acquired Pneumonia — right lower lobe	J18.9	Comorbidity
5	Peripheral neuropathy, diabetic	E11.40	Comorbidity

Clinical Summary

Mrs. Thornton, a 72-year-old female with a background of T2DM, hypertension, and CKD Stage 3, presented to A&E; on 09/11/2024 with a 3-day history of productive cough, dyspnoea on exertion, and markedly elevated blood glucose (CBG on admission: 24.3 mmol/L). She reported poor adherence to her insulin regimen over the preceding 2 weeks due to nausea. On examination: RR 22/min, SpO₂ 91% on air, T 38.4°C, BP 162/96 mmHg, HR 98 bpm. Chest auscultation revealed right basal crepitations. CXR confirmed right lower lobe consolidation consistent with community-acquired pneumonia. eGFR 38 mL/min/1.73m² (baseline ~42).

Management: IV Co-Amoxiclav 1.2g TDS for 48h, stepped down to oral Amoxicillin 500mg TDS on Day 3 following clinical improvement and CRP trend (admission 187 → Day 3 94 → discharge 31 mg/L). Insulin regimen reviewed by the Diabetes CNS; basal insulin dose increased from 18 to 24 units Lantus OD. Fluid balance monitored closely given CKD; total IV fluids restricted to 500ml NaCl 0.9% over 6h on Day 1.

Key Investigations

Test	Admission	Day 3	Discharge	Reference
HbA1c	91 mmol/mol	—	—	< 53 mmol/mol
CBG (fasting)	24.3 mmol/L	14.1 mmol/L	8.7 mmol/L	4.0–7.0 mmol/L
eGFR	38 mL/min	35 mL/min	39 mL/min	> 60 mL/min
Creatinine	158 µmol/L	168 µmol/L	151 µmol/L	45–90 µmol/L
CRP	187 mg/L	94 mg/L	31 mg/L	< 10 mg/L
WBC	14.2 × 10 ⁹ /L	10.8 × 10 ⁹ /L	8.1 × 10 ⁹ /L	4.0–11.0 × 10 ⁹ /L
Potassium	5.2 mmol/L	4.8 mmol/L	4.6 mmol/L	3.5–5.0 mmol/L
Sodium	131 mmol/L	136 mmol/L	138 mmol/L	135–145 mmol/L
Troponin T	18 ng/L	21 ng/L	19 ng/L	< 14 ng/L (borderline)
SpO2 (on air)	91%	95%	97%	> 95%

Discharge Medications

■ ALLERGY ALERT: Penicillin — rash (documented 2019). Metformin HELD — eGFR < 45. NSAIDs CONTRAINDICATED given CKD.

Medication	Dose	Route	Frequency	Duration / Notes
Amoxicillin 500mg	500 mg	Oral	TDS	Complete 7-day course (Days 1–7 post-discharge)
Insulin Glargine (Lantus)	24 units	SC	OD nocte	Increased from 18u — review in 4 weeks
NovoRapid (Insulin Aspart)	4–8 units	SC	TDS with meals	Sliding scale guidance provided
Amlodipine	10 mg	Oral	OD	Ongoing — BP target < 130/80
Ramipril	5 mg	Oral	OD	Monitor eGFR + K+ in 2 weeks
Furosemide	40 mg	Oral	OD morning	Review at 2-week follow-up
Atorvastatin	40 mg	Oral	OD nocte	Ongoing
Aspirin	75 mg	Oral	OD	Ongoing — gastroprotection with PPI
Omeprazole	20 mg	Oral	OD	Gastroprotection

Follow-Up & Discharge Plan

Action	Responsible	Timeframe
Bloods: U&E;, eGFR, glucose, FBC	GP / Paddington Surgery	Within 2 weeks
Diabetes CNS telephone review — insulin titration	Meridian Diabetes Service	Day 7 post-discharge
Cardiology OPD — HF optimisation, echo	Mr. T. Okafor, Cardiology	6 weeks
Nephrology OPD — CKD monitoring, AKI episode review	Dr. L. Srinivasan, Renal	8 weeks
Community nursing — wound check, daily insulin support	District Nursing Team	Daily × 5 days
Smoking cessation referral issued (patient smokes 8/day)	GP	At next contact
Repeat CXR to confirm pneumonia resolution	GP / Radiology	6 weeks

Clinician Sign-Off

Prepared by:	Dr. Samira Khalil, ST5 Internal Medicine GMC: 7654321	Date/Time:	15 Nov 2024 10:45
Countersigned:	Mr. James R. Hadfield, Consultant Physician GMC: 1234567	Bleep:	Bleep 4421
Copy to:	GP (Dr. Mehta) · Patient · Cardiology · Nephrology · Community Nursing Team	System:	EPR ref: MGH-2024-110934

Clinical Notes (Nursing Handover)

- 09/11 22:00 — Patient settled, sats improved to 94% on 2L O2. BM 18.2. IV access patent. Fluid balance -200ml. Night observations stable. — SN Okonkwo
- 10/11 06:30 — Overnight BMs: 22:00→18.2, 02:00→15.6, 06:00→12.4. IV AB given as prescribed. Patient slept well, no respiratory distress. — SN Okonkwo
- 11/11 14:00 — Stepped to oral ABX. Tolerating diet. BM pre-lunch 9.8. Physio reviewed — mobilising with zimmer. Wound on R) heel — dressing changed, Tissue Viability reviewed. — SN Davies
- 13/11 10:00 — Discharge planning commenced. DN referral sent. TTOs prepared, awaiting pharmacy. Patient educated on insulin self-administration — observed × 2 doses. — SN Patel
- 15/11 09:00 — Medically fit for discharge. All TTOs dispensed. Patient and daughter briefed on all medications. Community nursing confirmed for PM today. — CN Rodriguez

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